Clinical Practice Guidelines for Hypertension Management

Comprehensive clinical practice guideline covering ACC/AHA diagnostic staging, secondary hypertension etiologies, first-line antihypertensive drugs, and crisis protocols.

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1. Diagnostic Criteria & Blood Pressure Classification

According to the ACC/AHA guidelines, normal resting blood pressure is defined as systolic < 120 mmHg and diastolic < 80 mmHg. Stage 1 hypertension is defined as SBP 130-139 mmHg or DBP 80-89 mmHg. Stage 2 hypertension is defined as SBP >= 140 mmHg or DBP >= 90 mmHg based on an average of two or more properly measured seated BP readings on separate occasions.

2. Primary vs. Secondary Etiologies

Primary (essential) hypertension accounts for over 90% of cases and involves a multifactorial interplay of renal sodium excretion, sympathetic nervous system overactivity, and renin-angiotensin-aldosterone system (RAAS) dysregulation. Secondary causes: renal artery stenosis, primary aldosteronism, pheochromocytoma, and Cushing syndrome.

3. First-Line Pharmacotherapy

The four primary drug classes recommended include: ACE inhibitors (Lisinopril), Angiotensin II Receptor Blockers (Losartan), Calcium Channel Blockers (Amlodipine), and Thiazide-like diuretics (Chlorthalidone). In diabetic patients, ACEi/ARBs provide essential renal protection by dilating efferent arterioles.